

Patient Name: Paula Hall

DOB: 02/15/1953

Account Number: 86588

Patient Address: 1285 Angus Trl Lexington KY 40509 **DOS:** 05/04/2023 10:00 AM

Chief Complaint - Knee

Left Knee:

Left Knee Pain Present

Left Knee Pain Described As: Aching Sore Stiffness

The frequency of left knee pain is reported as daily.

Intensity of pain on a 0 to 10 VAS scale (0 being no pain and 10 being worst pain you have ever had) is reported to be 5/10 on VAS.

Left knee pain started gradually.

Patient reports left knee pain occurring for 10 ; year(s).

Right Knee:

Right Knee Pain Present

Right Knee Pain Described As: Aching Sore Stiffness

The frequency of right knee pain is reported as daily.

Intensity of pain on a 0 to 10 VAS scale (0 being no pain and 10 being worst pain you have ever had) is reported to be 5/10 on VAS.

Right knee pain started gradually.

Patient reports right knee pain occurring for 10; year(s).

HPI Knee

The patient presents today for evaluation and treatment of above complaints. Patient denies any radiating symptoms.

The patient has experienced this pain before. The patient reports crepitus on active motion in both the left and right knees. The patient reports morning stiffness in bilateral knees lasting less than 20-30 minutes. Patient's range of motion is decreased secondary to pain. Patient reports there was no trauma to the area. Patient reports using ibuprofen 400mg Q 4-6 hours as needed for six months and states no overall significant improvement. Patient reports using Tylenol 500mg BID as needed for six months and states no overall significant improvement. Patient reports not having tried muscle relaxants for management of symptoms. Patient reports not having tried narcotic pain medication for management of symptoms. Patient has tried cortisone injections most recent injection was four years ago and states no overall significant improvement. The patient attempted the following to manage symptoms: HEP for six months and states no overall significant improvement.

Improvement of Pain

HPI: Patient reports improvement of knee pain with the following:

Heat

OTC Cream

Worsening Pain

HPI: Patient reports worsening of knee pain with the following Activities of Daily Living:

Bending

In/ Out of Vehicle

Kneeling

Squatting/ Stooping

Stairs

Past Treatments v3

HPI: Patient reports having tried the following treatment in the past:

Heat

OTC Creams

Patient Allergies (v2)

Allergies: None

Patient Medications (v2)**Medications:**

None

Patient is currently not taking blood thinners.

Patient Family History (v2)

Family Medical History Inclusive of: Arthritis

Patient Medical History (v2)**Past Medical History:**

Past Medical History Inclusive of: Cancer Joint Pain Low Back Pain
MELANOMA YEARS AGO

Patient Social History**Social History:**

Alcohol Use History: Social Drinker

Smoking History: Never Smoker

Patient Surgical History (v2)**Surgical History:**

Surgical History Inclusive of: Hysterectomy

Vital Signs

Weight 169 lbs Height 5' 7", BMI 26.46 kg/m sq, Category

Knee testing for instability has been performed. Functional knee testing as noted below:

KNEE RADIOGRAPHIC FINDINGS REPORT

INDICATION: Knee pain

Arthritis Knee Pain Centers
1019 Majestic Dr
Ste 270 D
Lexington
KY
40513-1496

Knee Measurements

Left Knee Measurements

Thigh Measurement (in): 16

Knee Center Measurement (in): 14

Calf Measurement (in): 13

Right Knee Measurements

Thigh Measurement (in): 16

Knee Center Measurement (in): 13

Calf Measurement (in): 13

Knee Inspection v5

Physical Exam:

Left Knee Findings:

Varus Angulation Deformity

Right Knee Findings:

Varus Angulation Deformity

Palpation:

Left Knee Crepitus is present with palpation in left knee.

Right Knee Crepitus is present with palpation in right knee.

Left Knee Pain with Palpation

Patient exhibits left knee pain with palpation

Pain with palpation exhibited at Medial Joint Line

Severity of Pain Moderate

Right Knee Pain with Palpation

Patient exhibits right knee pain with palpation

Pain with palpation exhibited at Medial Joint Line

Severity of Pain Moderate

Palpable Warmth

Left Knee

No evidence of palpable warmth, erythema, or skin rash on today's exam.

Right Knee

No evidence of palpable warmth, erythema, or skin rash on today's exam.

Knee Range of Motion:

Left Knee

Range of Motion Decreased

Range of motion is limited secondary to pain in left knee.

Right Knee

Range of Motion Decreased

Range of motion is limited secondary to pain in right knee.

Gait and Ambulation:

Altered Gait

Patient ambulation status is described as ambulatory without assisted devices

Physical Exam

Knee Functionality

Left Knee: Positive Valgus Stress Test; Right Knee: Positive Valgus Stress Test

Left Knee: Positive Grinding Test; Right Knee: Positive Grinding Test

Diagnoses

Bilateral primary osteoarthritis of knee - M17.0

Unilateral primary osteoarthritis, right knee - M17.11

Unilateral primary osteoarthritis, left knee - M17.12

Pain in right knee - M25.561

Pain in left knee - M25.562

Stiffness of right knee, not elsewhere classified - M25.661

Stiffness of left knee, not elsewhere classified - M25.662

Difficulty in walking, not elsewhere classified - R26.2

Plan:

Patient has been recommended the following for bilateral knees:

Injectable:

The patient will be treated with a series of 3 viscosupplementation procedures for placement of Synvisc (J7325) in the knee. This will be performed at a frequency of once per week for 3 weeks. The patient will receive a dosage of 16 mg / 2 ml per knee for each procedure. FDA approved hyaluronic acid shows a statistically significant reduction in knee pain and increasing levels of mobility in the majority of patients treated when compared to placebo. Promising evidence shows that Synvisc and other forms of hyaluronic acid may slow deterioration of joints. To ensure that the medical device is placed properly into the joint space, fluoroscopic guidance will be necessary. This will allow for examination to assess the condition of the knee and help to rule out possible contraindications or other pathology causing or contributing to the patient's pain. In order to ensure the device is properly placed specifically into the knee capsule itself, subsequent double confirmation of the device will be necessary as per FDA guidelines. In order to avoid the delicate and possibly damaged and inflamed tissues surrounding the capsule, an initial diagnostic arthrogram will be performed. Fluoroscopic guidance for device placement, followed by a therapeutic arthrogram will be performed to diminish possibilities of the prosthetic medical device being placed anywhere other than into the knee capsule. This will help to diminish complications and lessen local flare reactions in the tissues surrounding the knee joint. Evidence shows that up to 40% of putative knee joint injections miss the joint space completely.

1. Jackson, Douglas W., et al. Accuracy of Needle Placement into the Intra-Articular Space of the Knee. The Journal of Bone and Joint Surgery(American) 84:1522-1527 (2002)

Reviewed benefits, risks and the following contraindications to treatment including but not limited to allergy to NSAIDs, renal impairment, uncontrolled hypertension, stomach ulcers, and uncontrolled anticoagulant therapy.

The patient will be treated with a series of 2- Ketorolac Tromethamine (Toradol) (J1885) injection procedures in the knee. This will be performed at a frequency of one-week intervals between each procedure. The patient will receive a dosage of 15mg / 1 ml per knee for each procedure. To ensure that proper placement is achieved into the intra-articular space, fluoroscopic guidance will be performed. This will help to diminish complications and lessen local flare reactions in the tissues surrounding the knee joint.

Following completion of Ketorolac Tromethamine (Toradol) (J1885) series the patient will return in one week to begin weekly viscosupplement treatment. Intra-articular viscosupplement injections with ketorolac showed more rapid analgesic onset than intra-articular viscosupplement injections alone when compared to the placebo group. Significant improvement regarding Oxford assessment tool was observed in addition when compared to viscosupplement treatment, further supporting improvement in pain and functionality.

Lee, S. C., Rha, D.-W., & Chang, W. H. (2011). Rapid analgesic onset of intra-articular hyaluronic acid with ketorolac in osteoarthritis of the knee. Journal of Back and Musculoskeletal Rehabilitation, 24(1), 31–38. <https://doi.org/10.3233/bmr-2011-0272>

Plan:

Patient has been recommended the following for the bilateral knees:

DME:

Patient has been prescribed a left and right VQ Ortho OActive 2 single upright unloading orthosis due to knee instability and ligamentous laxity seen on physical examination, orthopedic testing, and plain film radiography related to osteoarthritis. The orthosis will be utilized as adjunctive therapy in reducing the level of pain and symptoms associated with osteoarthritis of the knee, to stabilize the knee, for overall improvement of the knee. Improving function under weight bearing loads of the joint thereby increasing tolerance to ADL's.

Patient has been recommended the following for bilateral knees:

Patient has been discharged with a comprehensive HEP.

Summary and Assessment V2

Assessment::

Symptomatic osteoarthritis is observed

Inability to perform ADLs due to osteoarthritis

Range of motion decreased

Conservative therapy trialed and failed for at least 3 months: tylenol, IBU, HEP

The goals of our treatment plan are as follows::

Decrease Pain

Increase Endurance/ Stamina of Affected Area

Increase Pain Free Range of Motion

Stabilize the Joint

Additional Goals:

Increase ability to perform and tolerate ADLs

Follow Up:

Patient will return in 4-6 weeks following completion of treatment for reevaluation.

Knee X-Ray Findings/ Impression

The study was performed on: date of knee screening with the patient in a standing position. Radiographs were performed consisting of AP and lateral views of the knees bilaterally. Technical: The radiographs are of readable quality. The left knee presents with varus deformity on x-ray. The right knee presents with varus deformity on x-ray. Areas of bony sclerosis are found in the left and right knees. Bone density appears abnormal. Bony contour abnormalities are noted in the left and right knees. Osteophytes are noted on the medial joint line of the knees bilaterally. Sharpening of the tibial spines is noted bilaterally. No subchondral cyst formation visualized. No subluxation of tibia visualized. Mild to moderate narrowing is found in the medial compartment of the left knee. Mild to moderate narrowing is found in the medial compartment of the right knee. The left knee is rated at Kellgren-Lawrence Classification Grade II which is defined as: small osteophytes, mild to moderate joint space narrowing. The right knee is rated at Kellgren-Lawrence Classification Grade II which is defined as: small osteophytes, mild to moderate joint space narrowing. Radiological findings were reviewed at the time imaging was obtained.

Electronically signed by Bird, Shawna LeAnne, FNP. on Thursday, May 4, 2023 at 10:47 AM